TASK 4 – ADVERSE DRUG REACTION

SCENARIO 1 – METRONIDAZOLE (ANTIBIOTIC/ANTIPROTOZOAL)

Medicine Name: Metronidazole

Dose: 400 mg tablet, taken three times daily as prescribed.

Timeline: Started on October 1, 2026. Stopped on October 4, 2026, after contacting the prescriber.

Event: On October 3, 2026, the patient noticed a persistent metallic taste in the mouth. The prescriber considered a possible association with metronidazole and advised a medication review. The taste gradually resolved after the course was discontinued. No medical treatment or hospital admission was required. A metallic taste is a recognized adverse effect of metronidazole.

SCENARIO 2 – GABAPENTIN (ANTICONVULSANT/NEUROPATHIC PAIN AGENT)

Medicine Name: Gabapentin

Dose: 300 mg capsule, taken at night.

Timeline: Started on October 2, 2026. The dose was withheld on October 6, 2026, following consultation with the prescriber.

Event: On October 5, 2026, the patient developed dizziness and unsteadiness while walking. The prescriber suspected a possible association with gabapentin and reviewed the treatment. The symptoms improved after the regimen was adjusted. No hospital admission was required. Dizziness and impaired coordination are recognized adverse effects of gabapentin.

SCENARIO 3 – PREDNISOLONE (CORTICOSTEROID)

Medicine Name: Prednisolone

Dose: 10 mg tablet, taken once daily in the morning for a short prescribed course.

Timeline: Started on October 1, 2026. The prescriber reviewed the treatment on October 7, 2026.

Event: On October 6, 2026, the patient reported difficulty sleeping after starting prednisolone. The prescriber considered the medicine a possible contributor and reviewed the dose timing and treatment plan. The sleep disturbance improved after the regimen was adjusted. No hospital admission was required. Insomnia and sleep disturbance are known possible adverse effects of systemic corticosteroids.

SCENARIO 4 – DOXYCYCLINE (TETRACYCLINE ANTIBIOTIC)

Medicine Name: Doxycycline

Dose: 100 mg capsule, taken once daily as prescribed.

Timeline: Started on October 1, 2026. Discontinued on October 5, 2026, after medical review.

Event: On October 4, 2026, the patient developed increased sensitivity to sunlight and redness on exposed skin after outdoor activity. The clinician suspected a possible doxycycline-associated photosensitivity reaction and advised stopping or changing treatment as clinically appropriate, along with sun protection. The redness resolved over the next few days. No hospitalization was required. Photosensitivity is a recognized adverse effect of doxycycline.

SCENARIO 5 – FLUCONAZOLE (ANTIFUNGAL)

Medicine Name: Fluconazole

Dose: 150 mg capsule, taken as a single dose as prescribed.

Timeline: Taken on October 3, 2026. The patient sought advice on October 4, 2026.

Event: On October 4, 2026, the patient experienced mild nausea and abdominal discomfort. The healthcare professional considered fluconazole as a possible cause and recommended supportive care and monitoring. Symptoms resolved by October 5 without hospital admission. Nausea and abdominal discomfort are recognized possible adverse effects of fluconazole.

SCENARIO 6 – TRAMADOL (OPIOID ANALGESIC)

Medicine Name: Tramadol

Dose: 50 mg tablet, taken every 8 hours as needed, within the prescribed maximum.

Timeline: Started on October 8, 2026. Further doses were withheld on October 9, 2026, after contacting the prescriber.

Event: On October 9, 2026, the patient developed nausea and vomiting after taking tramadol. The prescriber suspected a possible medication-related reaction and reviewed pain management. Symptoms settled with appropriate advice and no hospital admission was required in this illustrative case. Nausea and vomiting are known adverse effects of tramadol.

SCENARIO 7 – FUROSEMIDE (LOOP DIURETIC)

Medicine Name: Furosemide

Dose: 20 mg tablet, taken once daily in the morning as prescribed.

Timeline: Started on October 1, 2026. Reviewed by the prescriber on October 6, 2026.

Event: On October 5, 2026, the patient reported light-headedness on standing. Clinical assessment found a lower-than-usual blood pressure, and the prescriber considered volume

depletion related to diuretic therapy as a possible cause. The dose and hydration plan were reviewed, and symptoms improved. No hospital admission was required. Dizziness and hypotension can occur with furosemide, particularly if fluid loss is excessive.

CLASSIFICATION OF SERIOUSNESS AND EXPECTEDNESS

SL NO.	MEDICINE	ADVERSE REACTION	SERIOUSNESS	EXPECTEDNESS	RATIONALE
1	**Metronidazole**	Metallic taste	Non-serious	Expected	No medical intervention or admission was required; metallic taste is a recognized adverse effect.
2	**Gabapentin**	Dizziness/unsteadiness	Non-serious in this example	Expected	Symptoms improved after review; dizziness and impaired coordination are recognized effects. Falls or severe symptoms would need further assessment.
3	**Prednisolone**	Insomnia/sleep disturbance	Non-serious	Expected	No admission was required; sleep disturbance is a known possible effect of systemic corticosteroids.
4	**Doxycycline**	Photosensitivity/redness	Non-serious in this example	Expected	Resolved without admission; photosensitivity is a recognized reaction. Severe blistering requires medical review.
5	**Fluconazole**	Nausea/abdominal discomfort	Non-serious	Expected	Symptoms resolved without admission; gastrointestinal effects are recognized.
6	**Tramadol**	Nausea and vomiting	Non-serious in this example	Expected	No admission was required; nausea and vomiting are known effects.

7	**Furosemide**	Postural light-headedness/low BP	Non-serious in this example	Expected	Improved after clinical review; hypotension/dizziness can occur with diuresis.

Reference: ICH E2A – Clinical Safety Data Management: Definitions and Standards for Expedited Reporting.